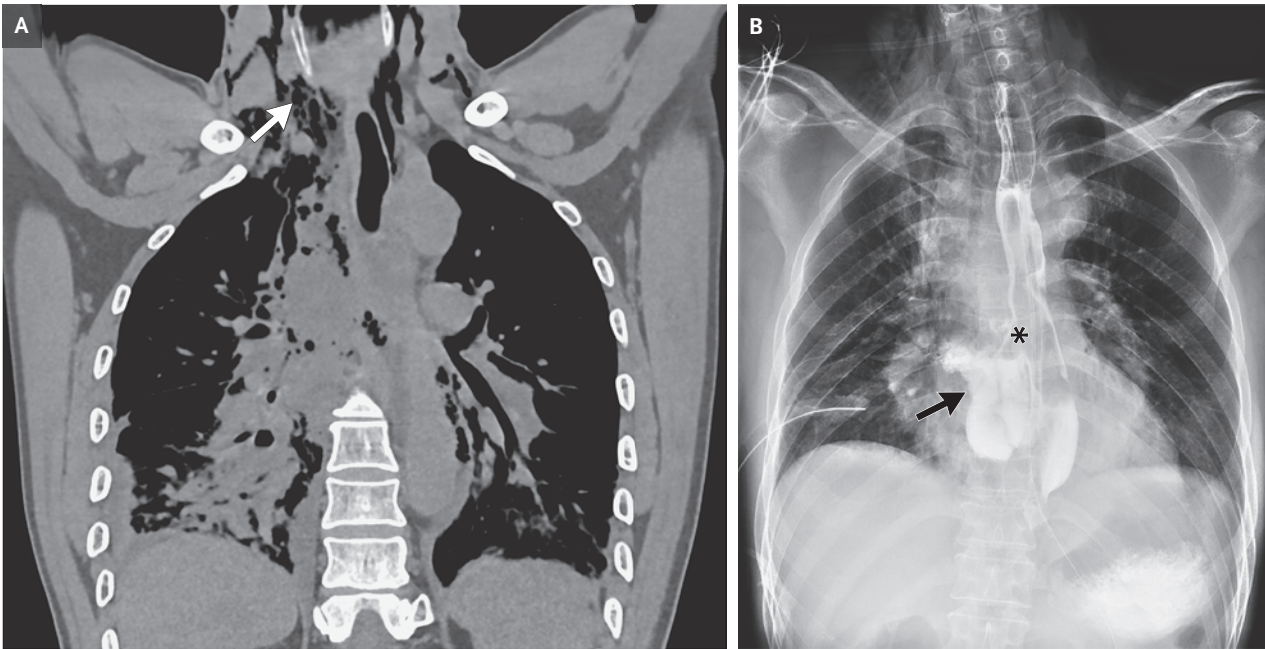


IMAGES IN CLINICAL MEDICINE

Boerhaave Syndrome



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A PREVIOUSLY HEALTHY 59-YEAR-OLD MAN PRESENTED TO THE EMERGENCY department with a 5-hour history of severe, pleuritic chest pain. Half an hour before the onset of symptoms, he had vomited a large amount of gastric contents after eating street food. On physical examination, his breathing was found to be rapid and shallow. Palpable crepitus was noted in the neck, and breath sounds at the base of the right lung were diminished. Computed tomography of the chest revealed air in the mediastinum extending up to the neck (Panel A, arrow), a perforation of the esophagus, a pleural effusion on the right side, and atelectasis in the right lung. A chest tube was inserted and exudative fluid without gastric contents was drained, which indicated that the effusion resulted from reactive pleuritis. An esophagram that was obtained with the use of water-soluble contrast material confirmed a large laceration of the esophagus (Panel B, asterisk) with extravasation of the contrast material into the mediastinum (Panel B, arrow). A diagnosis of the Boerhaave syndrome — life-threatening, spontaneous rupture of the esophagus owing to severe vomiting or retching — was made. Emergency thoracoscopic repair of the esophageal perforation was performed. The patient was hospitalized for 35 days after the procedure and was discharged with a nasojejunal feeding tube. At a 3-month follow-up visit, imaging showed that the esophageal perforation had resolved, and the feeding tube was removed.

DOI: 10.1056/NEJMicm2500300

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